

Guide for Outbreak Prevention & Control in Continuing Care Homes

Includes viral respiratory & gastrointestinal illness



If you have feedback about this guide email

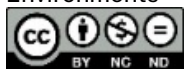
CDCResourceFeedback@primarycarealberta.ca.

If you have questions about a specific outbreak, or facility-specific processes, always direct your questions to your designated facility lead or the Public Health Outbreak Team.

Navigating this resource

- The most up-to-date version of the guide is the electronic version on the website. Printed copies of the guide should be considered current only on the date printed.
- Bolded terms are defined in the glossary.

© 2025 Primary Care Alberta, Communicable Disease Control and Alberta Health Services, Safe Healthy Environments



This copyright work is licensed under the [Creative Commons - Attribution-NonCommercial-NoDerivatives 4.0 International](https://creativecommons.org/licenses/by-nc-nd/4.0/). To view a copy of this license, see [Creative Commons - Attribution-NonCommercial-NoDerivatives 4.0 International - CC BY-NC-ND 4.0](https://creativecommons.org/licenses/by-nc-nd/4.0/). You are free to copy and distribute the work including in other media and formats for non-commercial purposes, as long as you attribute the work to Primary Care Alberta and Alberta Health Services, do not adapt the work, and abide by the other license terms. The license does not apply to Primary Care Alberta and Alberta Health Services trademarks, logos or content for which Primary Care Alberta and Alberta Health Services are not the copyright owner.

Disclaimer

This material is intended for general information only and is provided on an “as is”, “where is” basis. Although reasonable efforts were made to confirm the accuracy of the information Primary Care Alberta and Alberta Health Services does not make any representation or warranty, express, implied or statutory, as to the accuracy, reliability, completeness, applicability or fitness for a particular purpose of such information. This material is not a substitute for the advice of a qualified health professional. Primary Care Alberta and Alberta Health Services expressly disclaims all liability for the use of these materials, and for any claims, actions, demands or suits arising from such use.

Table of contents

Introduction	6
1. Outbreak prevention.....	8
1.1 Prevent the spread of illness.....	9
1.2 Use routine IPC practices	10
1.3 Isolate residents with symptoms or confirmed illness	12
1.4 Prevent spread of illness from HCW/staff.....	14
2. Identify and report viral respiratory illness outbreaks.....	16
2.1 Monitor and report viral respiratory illness symptoms.....	17
2.2 After reporting to PPHST	18
3. Viral respiratory illness outbreak control	19
3.1 Isolate residents with symptoms or confirmed illness	20
3.2 Communicate and coordinate	20
3.3 Complete outbreak environmental cleaning and disinfection.....	22
3.4 Plan safe activities for residents who are not on isolation	24
3.5 Plan safe visits during an outbreak	25
3.6 Admission, transfer and discharge restrictions	26
3.7 Transfer and discharge from an acute care facility	27
3.8 Use food service modifications in the outbreak facility/unit.....	28
3.9 Specimen collection	29
3.10 Control measures for complex outbreaks.....	29
3.11 End the outbreak	30
4. Identify and report gastrointestinal illness outbreaks	31
4.1 Monitor and report gastrointestinal illness symptoms	32
4.2 After reporting to PPHST	33
5. Gastrointestinal illness outbreak control	34
5.1 Isolate residents with symptoms or confirmed illness	35
5.2 Communicate and coordinate	35
5.3 Complete outbreak environmental cleaning and disinfection.....	36
5.4 Plan safe activities for residents who are not on isolation	39
5.5 Plan safe visits during an outbreak	40
5.6 Admission, transfer and discharge restrictions	41
5.7 Transfer and discharge from an acute care facility	41
5.8 Use food service modifications in the outbreak facility/unit.....	42
5.9 Specimen collection	42
5.10 Control measures for complex outbreaks.....	43
5.11 How to manage relapse gastrointestinal illness cases	43
5.12 End the outbreak	44
Glossary	46

Appendix A: Roles and Responsibilities	50
Appendix B: Resident Isolation for Viral Respiratory Illness.....	56
Appendix C: Case and Outbreak Definitions	57
Appendix D: ProvLab Specimen Collection Guidance.....	63
Appendix E: Influenza Outbreak Work Restrictions.....	65



Land acknowledgement

Our work takes place on historical and contemporary Indigenous lands, including the territories of Treaty 6, Treaty 7 & Treaty 8 and the homeland of the Métis Nation within Alberta and 8 Metis Settlements. We also acknowledge the many Indigenous communities that have been forged in urban centres across Alberta.

Introduction

Continuing care homes (Type A, B and C) are high-risk settings for the spread of communicable disease. Early detection of viral respiratory and gastrointestinal (GI) illness is important to reduce the spread of disease and prevent outbreaks¹. Although infectious disease outbreaks occur year-round, they are more common during fall and winter.

The notification of outbreaks and other infectious disease threats in Alberta is mandated under Section 26 of Alberta's [Public Health Act](#). Early recognition and rapid response are essential for effective outbreak prevention and management.

Outbreak management uses a multidisciplinary approach

[Appendix A](#) outlines the roles and responsibilities of:

- The **Public Health Outbreak team**
 - This includes Medical Officers of Health (MOH), Communicable Disease Control (CDC) Nurses and Safe Healthy Environments (SHE) Public Health Inspectors
- Primary Care Alberta (PCA) Provincial Partner Oversight (PPO) team
- **Infection Prevention & Control (IPC)**
- **Facility operator / facility management**
- **Workplace Health and Safety (WHS) / Occupational Health and Safety (OHS)**
- Onsite **Healthcare Worker (HCW) / staff**
- Public Health Laboratory (ProvLab)
- The **Outbreak Management Team (OMT)**

This guide was developed by CDC Primary Care Alberta (PCA) and SHE Alberta Health Services (AHS), in collaboration with the following partners:

- AHS:
 - Infection Prevention & Control
 - Workplace Health and Safety
- Alberta Primary and Preventative Health Services (PPHS), Office of the Chief Medical Officer of Health (OCMOH), Government of Alberta (GOA):
 - Medical Officers of Health
- Assisted Living Alberta (ALA):
 - Provincial Seniors Health and Continuing Care
- Covenant Health:
 - Infection Prevention & Control
 - Occupational Health, Safety & Wellness

¹ An outbreak occurs when there are more cases of a communicable disease than are normally expected in a specific time and place.

Best practice recommendations

This guide provides best practice recommendations and follows the Alberta Primary and Preventative Health Services (PPHS) [Public Health Disease Management Guidelines](#). It supports facilities to fulfill their obligation to prevent and control outbreaks. The guide is intended to be used in conjunction with requirements outlined in other provincial acts and standards and with facility policies.

Facilities are responsible to prepare for outbreaks

In compliance with the Alberta [Continuing Care Health Services Standards](#), AHS facilities, ALA facilities and contracted service providers will develop and implement documented policies and procedures for outbreak prevention, identification, management, control, surveillance and reporting of notifiable diseases (Standard 11, 2024).

Prior to outbreak season, the Public Health Outbreak team:

- Updates outbreak guides and supporting resources and posts on the Outbreak Management webpage at ahs.ca/outbreak.
- Provides annual outbreak zone education sessions to continuing care home representatives.
- Sends preparation letters to facilities and community partners outlining general responsibilities for outbreak preparation.

1. Outbreak prevention

This section includes best practice recommendations to prevent outbreaks. These are routine actions for every facility and every resident.



Key actions

- ➔ Prepare for outbreaks.
- ➔ Promote immunization of residents, HCW/staff, designated family / support person and visitors.
- ➔ Handle food safely.
- ➔ Facilitate safe visiting practices.
- ➔ Post signs to remind visitors to defer visit.
- ➔ Complete routine environmental cleaning and disinfection.
- ➔ Perform hand hygiene and respiratory etiquette.
- ➔ Conduct an Infection Prevention and Control Risk Assessment (IPCRA) prior to providing resident care.
- ➔ Isolate residents with symptoms or confirmed illness.
- ➔ Offer antiviral treatment if the pathogen is COVID-19 or influenza.
- ➔ Prevent illness spread by monitoring for symptoms of illness and not attending work when ill.
- ➔ Restrict HCW/staff with symptoms or confirmed illness from attending work.

Supporting resources

- [Outbreak Prevention Checklist for Continuing Care Homes](#)

1.1 Prevent the spread of illness

These routine everyday practices are essential to stop the spread of viral respiratory and GI illnesses that cause outbreaks. Outbreak prevention practices support a healthy environment for residents, HCW/staff, DFSP and visitors.

Prepare for outbreaks

Facilities are responsible to prepare for outbreaks. This allows for a rapid response and early intervention before illness spreads.

Make a plan for outbreaks

- For detailed direction for outbreak planning and preparation prior to an outbreak see [Appendix A: Roles and Responsibilities](#).
- This includes making a facility outbreak management plan and an influenza antiviral chemoprophylaxis plan for residents and HCW/staff. Refer to *Minimum Requirements of a Comprehensive Communicable Disease Emergency Response Plan* in [Continuing Care Communicable Disease Emergency Response Guide 2024](#).

Communicate and share information

- Attend annual fall outbreak zone education sessions.
- Provide HCW/staff with an annual review of:
 - The facility outbreak management plan
 - The Guide for Outbreak Prevention & Control in Continuing Care Homes and supporting resources posted at ahs.ca/outbreak.
- Review and distribute any preparation letters sent by the Public Health Outbreak team to residents and community partners outlining general responsibilities for outbreak preparation.

Stock adequate supplies

- Stock non-expired outbreak management supplies including PPE and specimen collection kits.

Promote immunization of residents, HCW/staff, designated family / support person (DFSP) and visitors

- Immunization is the best way to protect residents, HCW/staff, **designated family / support person (DFSP)** and **visitors** from vaccine-preventable diseases. Refer to [Recommended Vaccines](#).
- The PCA Provincial Partner Oversight (PPO) team supports facilities in providing annual influenza and COVID-19 immunization for residents and HCW/staff, as well as pneumococcal and RSV immunization for residents who are eligible.

Handle food safely

- Follow routine safe food handling practices to reduce the spread of illness by contamination.

- Keep raw meat, poultry, eggs, fish and shellfish away from other foods/surfaces that may come into contact with ready-to eat foods.
- To further separate raw and ready-to-eat foods, use separate cutting boards and preparation areas if possible, and frequent kitchen cleaning/disinfection.
- Wash hands well and often, especially before and after preparing foods, after handling raw meat and when they become contaminated.
- Keep perishable food in the safe temperature zones and use within appropriate timeframes.
- Store pantry items to prevent contamination. Use a “first in first out” approach.
- Refer to information from Environmental Public Health for Food Facilities at [Information for Your Business](#).

Facilitate safe visiting practices

Help plan a safe visit

- Refer to [Family/Visitors of Patients and Residents](#).
- Request DFSP and visitors follow facility directions.
- Post signage to remind DFSPs and visitors to defer visit if they are ill.
- Request that DFSPs and visitors reschedule visit if they are ill.
 - Determine if exemption will be made due to extenuating circumstances.
 - Offer compassionate exemptions for visits to residents who are at end of life.
- Advise DFSPs and visitors to perform hand hygiene and [respiratory etiquette](#).
 - [Using alcohol-based hand rub](#)
 - [Washing hands with plain soap](#).

How to safely visit residents who are on isolation

Provide the following messages to DFSPs and visitors:

- There is a risk of exposure to illness.
- Use personal protective equipment (PPE) as per additional precautions signage.
 - Demonstrate how to use PPE.
- Use physical distancing if not wearing PPE while visiting residents with viral respiratory illness.
- Only visit the resident who is on isolation and then leave the facility promptly.

1.2 Use routine IPC practices

Implementing [routine IPC practices](#) is essential to prevent the spread of illness from person to person.

- Use routine IPC practices when caring for every resident, every time.
- Click on the Continuing Care tab on the [IPC Resource Manuals](#) web page for more information.
- Support for PPE selection and use is available. Refer to the AHS [Provincial PPE Safety Coach Program](#).

Complete routine environmental cleaning and disinfection

- Perform routine cleaning and disinfection. A clean environment protects residents and HCW/staff from infection by removing germs from environmental surfaces. It is one of the most effective ways to stop the spread of illness.
- **Cleaning** refers to using soap or detergent to remove visible dirt, grime, and impurities. Cleaning does not kill germs but helps remove them from the surface.
- **Disinfection** refers to using chemicals to kill germs on surfaces.

Steps for cleaning and disinfection

If using separate products for cleaning and disinfection:

- Use a “wipe twice” procedure (two-step process) to clean and then disinfect
 - First: Clean surfaces using a cleaning product such as soap or detergent, paying special attention to removing any visible dirt.
 - Second: Wipe again with a clean cloth saturated with a product that is an approved disinfectant.

If using a product approved for use as a one-step cleaner-disinfectant:

- If the surface is visibly clean, the product may be used without a pre-cleaning step. Follow the manufacturer’s instructions.
- If the surface is not visibly clean, first clean the surface and then disinfect using a “wipe twice” (two-step) process. Follow the manufacturer’s instructions.

- Follow [Public Health Recommendations for Environmental Cleaning and Disinfection of Public Facilities](#) for direction on:
 - How to clean and disinfect
 - Frequency of cleaning and disinfection
 - What products to use
 - Cleaning during viral respiratory and GI illness outbreaks.
- Follow [Principles for Environmental Cleaning and Disinfection](#) for direction on cleaning the environment and non-critical medical devices / equipment where care is provided.
- Follow [Linen in Community-based Services](#) for laundry instructions.
- Follow direction in [Laundering Personal Patient Items in Washing Machine](#).

Perform hand hygiene

- Hand hygiene (washing or sanitizing your hands) is the most effective way to prevent the spread of illness.
- [Using alcohol-based hand rub](#) when performing hand hygiene except when **plain soap** and water is recommended.
- [Washing hands with plain soap](#) and water is recommended:
 - When hands are visibly soiled with food, dirt, or blood and body fluids
 - Before, during, and after handling food
 - When removing gloves after caring for a resident with vomiting and/or diarrhea

- Immediately after using the washroom.
- Perform hand hygiene before and after glove removal. Glove use is not a substitute for hand hygiene.
- Follow the AHS [Hand Hygiene Policy and Procedure](#).

Perform respiratory etiquette

- [Cover coughs and sneezes](#) with a sleeve or tissue.
- Dispose used tissues in the garbage.
- Perform hand hygiene after coughing or sneezing.

Conduct an IPCRA prior to providing resident care

- Refer to the [IPCRA](#) to assess the task, the resident, and the environment prior to each resident interaction.
- Use [PPE](#) including an **appropriate mask**, eye protection, gown and gloves as indicated by the IPCRA.

1.3 Isolate residents with symptoms or confirmed illness

Immediately isolate any resident with symptoms or confirmed illness (the resident has tested positive for a viral respiratory or GI pathogen).

- Do not wait for a pathogen to be identified through specimen collection if the resident has symptoms.
- Consult **Infection Control Practitioner (ICP) / Infection Control Designate (ICD)** for additional support.

Resident accommodation

- A private room is preferred.
- Use a dedicated washroom or commode when the room is shared.
 - If not available, clean and disinfect the commode or washroom between use.
- Place isolation carts outside of the resident's room.
- Post additional precautions [signage](#) outside the door of the resident's room to alert HCW/staff, DFSP and visitors that additional precautions are required.

Use additional precautions based on symptoms

For viral respiratory symptoms

- Isolate using [Droplet and Contact precautions](#). For duration of isolation refer to [Appendix B: Resident Isolation for Viral Respiratory Illness](#).

For GI symptoms

- Isolate residents until 48 hours after the last episode of vomiting and/or diarrhea.
 - Diarrhea only: Use [Contact precautions](#).
 - Vomiting with or without diarrhea: Use [Droplet and Contact precautions](#).

How to care for residents who are on isolation

Maintain distance from other residents

- Provide meal service to the resident in their room and assist resident to perform hand hygiene before their meal.
- Provide a mask to a resident with viral respiratory symptoms if they need to leave their room.
- Delay participation in group activities until the isolation period has ended.
- Provide treatment such as physiotherapy in the resident's room.

Facilitate medically necessary appointments using additional precautions

- If not possible to attend a medically necessary appointment arrange a virtual visit.
- Notify the receiving provider so that precautions can be taken.
- For transfers to an **acute care** facility, notify the EMS dispatcher, the transport staff (EMS crew) and the acute care facility.

Support social engagement for residents who are on isolation

- Provide activities that engage/support the resident such as social, spiritual care and mental health.
- Support safe visits with DFSP and visitors.
- Provide one-on-one support for residents who need to leave their room.
 - Use strategies to minimize spread of infection such as wearing a mask, using hand sanitizer, maintaining distance from others and avoid touching surfaces.
- Residents with dementia or cognitive impairment may require additional engagement and support.

Share spaces and equipment safely

- Maintain at least two metres of distance between bed spaces of residents who are on isolation and sharing a room.
- Use physical barriers such as curtains or portable wipeable screens in shared rooms. Refer to [Additional Precautions Without Walls in Shared Patient Care Space](#).
- Dedicate care equipment to a single resident.
- If equipment must be shared between residents, clean and disinfect after each use.

After the isolation period ends

- Discontinue precautions.
- Request the resident bathe or shower after completing isolation for GI illness.
- Complete a post isolation cleaning and disinfection of the resident's room.
- Remove isolation supplies and signage.

Antiviral treatment for residents

Early initiation of antiviral treatment for residents who are eligible is critical for effective treatment.

COVID-19



If the pathogen is COVID-19

- The **most responsible health practitioner** is accountable to determine eligibility and prescribe treatment.
- Refer to [Outpatient Treatment for COVID-19](#).

Influenza



If the pathogen is influenza

- The most responsible health practitioner is accountable to determine eligibility and prescribe treatment.
- Refer to:
 - Health Canada [Flu \(influenza\): For health professionals](#) for identification of potential cases of influenza and indications for early antiviral treatment.
 - Influenza antiviral treatment resource: [Association of Medical Microbiology and Infectious Disease \(AMMI\) Canada](#).

1.4 Prevent spread of illness from HCW/staff

Follow employer work restrictions and requirements

- HCW/staff are required to follow organizational policies and procedures regarding hand hygiene, work attendance, masking, and eye protection.
- HCW/staff employed by AHS/ALA:
 - Follow AHS/ALA policies on hand hygiene, work attendance, work restriction, masking and eye protection including the [Attendance at Work and Respiratory Virus Symptoms Policy](#).

HCW/staff monitor for symptoms of illness

- Do not attend work when ill.

Viral respiratory illness symptoms

- Any one of the following symptoms that are new or worsening and not related to a pre-existing illness or health condition:
 - Fever or chills
 - Runny or stuffy nose
 - Sore throat
 - Cough
 - Difficulty breathing or shortness of breath
 - Loss or altered sense of taste/smell.

GI illness symptoms

- New onset of vomiting and/or diarrhea.

If symptoms develop at work

Symptoms	Action
Viral respiratory illness	• Perform hand hygiene and respiratory etiquette. • Notify manager and leave the workplace as soon as possible.
GI illness	• Perform hand hygiene. • Notify manager and leave the workplace as soon as possible. ◦ Manager to direct environmental cleaning (with PPE) of any washroom facilities used by the HCW/staff while ill.

Implement work restrictions for HCW/staff with symptoms or confirmed illness

Do not report to work with viral respiratory illness symptoms

- Stay away from work for at least 24 hours after all the following conditions are met:
 - Viral respiratory illness symptoms have improved,
 - Fever-free without the use of fever-reducing medication, and
 - No new viral respiratory illness symptoms have developed.
- Returning to work where there is contact with others:
 - HCW/staff may still be potentially infectious. It is recommended to continuously mask and perform thorough hand hygiene for five calendar days, starting from the first day HCW/staff are eligible to return to work.

Do not report to work with GI illness symptoms

- Stay away from work until at least 48 hours after last episode of vomiting and/or diarrhea.

Report illness

- Report illness to the manager/designate.
- Report to WHS/OHS if symptoms are related to workplace exposure.
 - Follow direction provided by WHS/OHS regarding additional work restrictions and requirements.

2. Identify and report viral respiratory illness outbreaks

This section includes information on identifying and reporting viral respiratory illness outbreaks, including COVID-19, influenza and influenza-like illness (ILI).

Refer to [4. Identify and report gastrointestinal illness outbreaks](#) for information on GI illness outbreaks.



Key actions

- ➔ Monitor residents for symptoms of illness.
- ➔ Determine if residents meet criteria for a viral respiratory illness case.
- ➔ Keep track of viral respiratory illness cases.
- ➔ Report to the Provincial Public Health Support Team (PPHST) if there are two or more viral respiratory illness cases within a seven-day period.
- ➔ Inform administrative staff that a report was made to PPHST.

Supporting resources

- [Outbreak Prevention Checklist for Continuing Care Homes](#)
- [Surveillance Case Tracking Sheet](#)

2.1 Monitor and report viral respiratory illness symptoms

Monitor for viral respiratory illness symptoms

Monitor for and keep track of residents with symptoms or confirmed illness (cases). This is called surveillance.

- A Surveillance Case Tracking Sheet is available for facilities to record:
 - Resident name(s)
 - Symptoms
 - Date symptoms started.

Identify viral respiratory illness cases

Count resident as a viral respiratory illness case if they have either:

- A positive test² for pathogens in Appendix B: Resident Isolation for Viral Respiratory Illness OR
- Any of the following new or worsening symptoms:
 - Fever³
 - Cough
 - Shortness of breath (SOB)
 - Sore throat
 - Runny nose / nasal congestion
 - Loss of taste and/or loss of smell
 - Decrease in oxygen (O₂) saturation level or increased O₂ requirement
 - Nausea/diarrhea³.

When to report

Report as soon as there are two or more resident viral respiratory illness cases within a seven-day period.

² Specimen collection is not required prior to reporting to PPHST. Facilities may determine that specimen collection is warranted for the diagnosis and medical management of residents with symptoms, including treatment with oseltamivir (Tamiflu) for influenza or Paxlovid for COVID-19. This testing is directed by the residents most responsible health practitioner.

³ A resident may develop fever, nausea and/or diarrhea following immunization with COVID-19 or influenza vaccine. The resident will not count as a surveillance case if:

- Onset is within 24 hours of being immunized AND
- They have no other viral respiratory illness symptoms AND
- Fever, nausea and/or diarrhea resolve within 48 hours of onset.

How to report



Call **1-844-343-0971** to report to the PCA Provincial Public Health Support Team (PPHST).

PPHST is a provincial, centralized outbreak reporting and response team. They provide initial outbreak support and direction to facilities.

PPHST will ask for facility details and operational information

- Facility name, address, phone number
- Caller name and contact information
- Email and phone number
- Facility type
- Number of units / unit names
- Number of residents and HCW/staff on affected units
- Total number of residents and HCW/staff in facility
- Number of symptomatic residents and HCW/staff
- Symptoms
- Initial Onset Date of symptoms
- Number of hospitalizations or deaths

Inform that a report was made to PPHST

- Make internal notifications to the following facility staff after reporting to PPHST:
 - Management
 - ICP/ICD
 - WHS/OHS.

2.2 After reporting to PPHST

PPHST notifies the Public Health Outbreak team

- After the facility report is made, PPHST will:
 - Send a summary of the report to the Public Health Outbreak team.
 - Advise facility when to expect a call from the Public Health Outbreak team.
- If the Public Health Outbreak team has not contacted the facility within 24 hours of the expected response time, the facility may call the CDC Intake Line at 1-888-522-1919.

The Public Health Outbreak team contacts the facility

The Public Health Outbreak team will determine if the facility meets the criteria for an outbreak using [Appendix C: Case and Outbreak Definitions](#). They will review the initial report and:

- Ask if there are additional viral respiratory illness cases since the initial report.
- Determine if cases are epidemiologically linked.

If an outbreak is opened, an email is sent to the facility providing ongoing contact information and instructions for how to complete daily outbreak reporting via the Facility CDC Outbreak Daily Report Portal (REDCap).

3. Viral respiratory illness outbreak control

This section includes best practice recommendations for the management of viral respiratory illness outbreaks including COVID-19, influenza and influenza-like illness (ILI). Non-viral respiratory pathogens such as bacterial and fungal pathogens may be responsible for illness **clusters** or outbreaks. Although some measures outlined in this section may be applicable in preventing or controlling them, it is beyond the scope of this guide, due to their unique epidemiological properties.

For GI illness outbreak control, refer to [Section 5. Gastrointestinal illness outbreak control](#).



Key actions

- ➔ Isolate residents with symptoms or confirmed illness.
- ➔ Implement continuous masking and eye protection on the outbreak unit for HCW/staff.
- ➔ Inform HCW/staff, residents, family/guardians, DFSP, visitors and partners of the outbreak.
- ➔ Identify and report new viral respiratory illness cases daily.
- ➔ Coordinate HCW/staff assignments.
- ➔ Complete outbreak environmental cleaning and disinfection.
- ➔ Plan safe activities for residents who are not on isolation.
- ➔ Plan for safe visits.
- ➔ Follow admission, transfer and discharge restrictions.
- ➔ Facilitate admissions and transfers from an acute care facility.
- ➔ Initiate outbreak modifications for food service.
- ➔ Collect specimens as directed.

Influenza outbreaks:

- ➔ Administer influenza antiviral chemoprophylaxis to residents.
- ➔ Offer influenza antiviral chemoprophylaxis to HCW/staff without adequate protection.

Supporting resources

- [Viral Respiratory Illness Outbreak Checklist for Continuing Care Homes](#)
- [Visiting During Viral Respiratory Illness Outbreaks](#)

Facilities are encouraged to contact the Public Health Outbreak team for advice and support with the recommendations in this section.

3.1 Isolate residents with symptoms or confirmed illness

How to isolate residents

- Use [Droplet and Contact precautions](#).
- Refer to [1.3 Isolate residents with symptoms or confirmed illness](#).

How long to isolate residents

- Refer to [Appendix B: Resident Isolation for Viral Respiratory Illness](#) for duration of isolation.

3.2 Communicate and coordinate

Implement continuous masking and eye protection for HCW/staff

- Wear an appropriate mask and eye protection:
 - In any resident space or common areas in the outbreak unit
- During indirect activities such as when preparing meals or doing laundry in the outbreak unit.

Inform HCW/staff, residents, family/guardians and partners of the outbreak

- Use established communication channels to notify:
 - Residents and families/guardians
 - Departments/partners inside and outside of the facility.
- Request HCW/staff such as nursing, allied health, food services, environmental services, recreational services and spiritual care to initiate **outbreak control measures**.
- Notify laundry services of the increased need for supplies.
- Post [outbreak signs](#) at the facility/unit entrance.

Identify and report new viral respiratory illness cases daily

- Maintain heightened surveillance to identify new viral respiratory illness cases.
- Request HCW/staff complete twice daily self-assessment for viral respiratory illness symptoms.
 - Direct HCW/staff to not attend work when ill and report symptoms of viral respiratory illness to the facility/unit manager/designate.
 - Notify WHS/OHS if symptoms are related to a workplace exposure.
- Report viral respiratory illness cases including hospitalized or deceased residents.
 - Review the email sent by the Public Health Outbreak team for how to report new cases using the Facility CDC Outbreak Daily Report Portal (REDCap).

Implement work restrictions for HCW/staff

- Refer to [1.4 Implement work restrictions for HCW/staff with symptoms or confirmed illness](#)

Coordinate HCW/staff assignments

- Consult with the Public Health Outbreak team when making decisions about HCW/staff assignments.
 - Direct HCW/staff to care for residents who are asymptomatic before residents with symptoms or confirmed illness.
 - Cohort HCW/staff to work only in affected areas or only in unaffected areas.
 - Minimize movement of HCW/staff between units, especially if some areas are not affected.
 - Assign HCW/staff that are immunized against the outbreak pathogen to care for residents with symptoms or confirmed illness.
- Refer to [Congregate Living Settings - Recommendations for Staff Cohorting](#)
- Consult with facility ICP/ICD, or the Public Health Outbreak team if considering **cohorting** residents.
- Refer to [Congregate Living Settings - Recommendations for Cohorting Clients](#)
- Assign HCW/staff to only housekeeping duties, or food preparation/service. If this is not possible, request staff complete any food preparation/service tasks before beginning housekeeping duties.

Influenza



If the outbreak pathogen is influenza.

- If HCW/staff is work restricted and works in more than one location, HCW/staff informs other work locations of the outbreak to determine if they may work in other settings.
- Refer to [Appendix E: Influenza Outbreak Work Restrictions](#).

Offer influenza antiviral chemoprophylaxis to HCW/staff who are not immunized

Influenza



If the outbreak pathogen is influenza

- Refer to [Appendix E: Influenza Outbreak Work Restrictions](#) for management of HCW/staff who are not immunized.
- The Public Health Outbreak team may supply letters to the facility/unit that outline options for HCW/staff who are not immunized.

Administer influenza antiviral chemoprophylaxis to residents

Influenza



If the outbreak pathogen is influenza.

- The Public Health Outbreak team will advise when to start and stop oseltamivir (Tamiflu) influenza antiviral chemoprophylaxis.
- Offer all asymptomatic residents influenza antiviral chemoprophylaxis regardless of immunization status.
- Residents will remain on chemoprophylaxis for the duration of the outbreak.
 - The minimum duration for residents to take antivirals is at least 10 days. Some residents may need to continue taking antivirals after the outbreak is over to complete the 10-day course.

Implement the site influenza antiviral chemoprophylaxis plan for Continuing Care Homes Type B

- Site operator/designate notify residents to initiate influenza antiviral chemoprophylaxis.
- Track residents with advanced prescriptions using the [Outbreak Antiviral Prophylaxis in Supportive Living Accommodations and Continuing Care Homes Type B Site Worksheet](#).
- Notify residents who have an advanced prescription to have it filled.
- Notify residents who did not get an advanced prescription to request one from a prescribing pharmacist, physician or nurse practitioner.

Prescribing influenza antiviral chemoprophylaxis

- The resident's most responsible health practitioner is accountable for prescribing influenza antiviral chemoprophylaxis.
- Refer to [Association of Medical Microbiology and Infectious Disease \(AMMI\) Canada](#) resources on influenza for chemoprophylaxis dosing recommendations.

Mixed outbreaks

- The Public Health Outbreak team will advise when to start and stop influenza antiviral chemoprophylaxis for mixed outbreaks that include influenza.

3.3 Complete outbreak environmental cleaning and disinfection

Increase cleaning and disinfection frequency in all areas⁴

Clean and disinfect visibly dirty surfaces immediately. Prioritize high-traffic areas.

- Follow [Public Health Recommendations for Environmental Cleaning and Disinfection of Public Facilities](#) during viral respiratory illness outbreaks.

⁴ Increased cleaning and disinfection are not required in settings such as private offices and administrative areas. Use routine cleaning and disinfection practices.

Areas to clean and disinfect	Frequency
• Low touch surfaces such as shelves and windowsills • Resident personal equipment such as walkers and wheelchairs.	At least once daily and when visibly dirty
• High touch surfaces such as doorknobs, light switches, handrails, phones, and elevator buttons • Care/treatment areas, including PPE carts. • Equipment such as computer keyboards, mouse, screens, desks, telephones, and touch screens.	At least twice daily and when visibly dirty
• Dining areas and lounges • High touch table and chair surfaces, including the underneath edge of the chair seat and table.	After each use
• All affected areas	At the end of the outbreak

Use a disinfectant that kills respiratory viruses

- Use a disinfectant with a drug identification number. Ensure it has a broad spectrum virucidal claim, or a specific virucidal claim against non-enveloped viruses and coronaviruses.
 - Refer to manufacturer instructions for product-specific cleaning and disinfection procedures, including compatible cleaners and disinfectants and contact time.
 - Use the product Safety Data Sheet for safety information.
- Alternatively, create a 1000 parts per million bleach solution by mixing 20 mL (four teaspoons) of unscented household bleach with 1000 mL (four cups) of water.
 - Store bleach solution in an opaque container and make a fresh solution at least every 24 hours. Bleach rapidly degrades in the presence of light and when mixed with water. Label container with date and time.
 - To effectively kill viruses, keep surfaces wet with the bleach water solution for at least one minute. Rinse food contact surfaces with clean water after cleaning with bleach.

Verify the concentration of the disinfectant

- When mixing disinfectants onsite, always verify the concentration using the test strips provided by the manufacturer.
- Test concentration daily.

Clean and disinfect rooms of residents who are on isolation

- Wear PPE and use [Droplet and Contact precautions](#).
- Clean and disinfect:
 - Resident room, moving from clean to dirty. Clean bathroom last.
 - Resident equipment such as wheelchairs and walkers according to

- manufacturer instructions.
 - Change linens (bed linens and towels)
- Change mop head (dry/wet), cloths and cleaning solution after cleaning each room for any resident who is on isolation.
- When additional precautions are discontinued:
 - Clean and disinfect the resident's room and equipment.
 - Discard disposable resident-care items.

Clean and disinfect shared rooms and equipment

- First clean and disinfect bedspaces of residents who are not on isolation.
- Then clean and disinfect bedspaces of residents who are on isolation.
 - Change mop head (dry/wet), cloths and cleaning solution after cleaning each bedspace.
- Clean and disinfect shared healthcare equipment such as commodes, blood pressure cuffs, thermometers, lifts, bathtubs, showers, and shared bathrooms after use and prior to use by another resident.
- Clean and disinfect equipment only with a product listed in the manufacturer instructions. Follow the procedures outlined by the manufacturer.

3.4 Plan safe activities for residents who are not on isolation

Type of activity	Action
Low risk resident activities	• Use physical distancing and hand hygiene for low-risk group activities such as art class, bingo and movies. • Limit personal services such as hairstyling to one resident at a time. • Encourage residents to wear a mask.
High risk resident activities	• Consult with the Public Health Outbreak team to determine if it is necessary to postpone/cancel high-risk group activities such as: ○ Singing and bus outings ○ Groups of residents who would not normally have contact with each other (mixing of residents from multiple units/floors). • Postpone/cancel previously scheduled resident events, such as holiday meals, parties, entertainers, school groups and presentations.
Non-resident activities	• Postpone/cancel any non-resident events booked for areas in the outbreak facility/unit such as in-person meetings.
Medical appointments	• Permit all appointments for residents who are asymptomatic. ○ Request the resident wear a mask. ○ Notify the receiving provider of the outbreak. ○ Virtual visits may be considered.

Type of activity	Action
Transfers to acute care	• If any resident from the outbreak facility/unit requires acute medical attention or treatment at an acute care facility, notify the following so that precautions may be taken: ○ EMS dispatcher and/or transport staff (for example the EMS crew) ○ Receiving provider.
Adult day programs	Before determining that a program may continue, the outbreak management team (OMT) and the Public Health Outbreak team confirm the following: • It is physically separate from areas of the facility in which there have been residents with symptoms or confirmed illness. • Residents attending the adult day program do not socialize with residents from the outbreak facility/unit. • Adult day program HCW/staff do not provide care in the areas of the facility in which there have been outbreak cases.

3.5 Plan safe visits during an outbreak

- Continue to follow safe visiting practices in [1.1 Help plan a safe visit](#).
- Provide DFSP and visitors with safe visiting recommendations during an outbreak.
 - [Visiting During Viral Respiratory Illness Outbreaks](#) resource is available for facilities to provide to DFSP and visitors.

Safe visiting recommendations during an outbreak

Visiting residents who are on isolation:

- Refer to [1.1 How to safely visit residents who are on isolation](#).
- Request that DFSP and visitors reschedule visit if they are ill.
 - Determine if exemption will be made due to extenuating circumstances.
 - Offer compassionate exemptions for visits to residents who are at end of life. Create a safety plan for visitation.

Provide the following messages to DFSP and visitors

Before the visit:

- Reschedule the visit if ill.
- Contact facility before arriving to determine if there are access limits.

During the visit:

- Check with reception prior to visiting any resident.
- Wear a mask in common areas on outbreak units.
- Minimize movement throughout the facility.
- Limit the number of individuals visiting a resident at the same time.

Safe visiting recommendations during an outbreak

- Limit visit to one resident only.

After the visit:

- Exit the facility immediately.

Additional restrictions may be implemented by the facility.

- A facility/unit may choose to limit visitation further than above in accordance with applicable organizational policy and in consultation with the Public Health Outbreak team.
- Consider the impact of the decision on resident and family well-being.

3.6 Admission, transfer and discharge restrictions

Determine if the outbreak facility/unit is open or restricted

- The ability of the outbreak facility/unit to accept admissions, transfers and discharges (whether it is open or restricted) is determined by the Public Health Outbreak team at the time the outbreak is opened.
- The scope of restrictions depends on:
 - The extent of the outbreak activity within the facility (for example, one unit/floor/wing or the entire facility)
 - The ability to cohort HCW/staff to affected areas
 - The severity of the outbreak (for example, new cases continue to develop despite implemented control measures).
- Consult the Public Health Outbreak team if issues related to admission, discharge and transfers arise during an outbreak.

Status	Action
Open	• Admissions, transfers, and discharges may proceed following usual non-outbreak processes.
Restricted	• Pause or delay admissions, transfers and discharges. This includes admissions to the outbreak affected facility/unit and transfers from the affected facility/unit to a different facility while restrictions are in place. ○ Implementation of restrictions may not be possible due to resident circumstances or operational need (including bed pressures). ○ If a facility with restrictions believes that an admission, transfer, or discharge is recommended to proceed despite restrictions, the facility follows the Risk Assessment Matrix and completes a Risk Assessment Worksheet.

Pathogen-specific restrictions to admission, transfer and discharge on the outbreak facility/unit

Restrictions depend on the outbreak pathogen.

COVID-19 	If the outbreak pathogen is COVID-19 • Admissions, transfers, and discharges are restricted. • Admission restrictions remain in place for the duration of the outbreak or as directed by the Public Health Outbreak team.
Influenza 	If the outbreak pathogen is influenza • Admissions, transfers, and discharges are restricted. • Admission restrictions remain in place for the duration of the outbreak or as directed by the Public Health Outbreak team.
ILI 	If the outbreak pathogen is influenza-like illness (ILI) • Admissions, transfers, and discharges generally remain open unless given different direction by the Public Health Outbreak team. • Admissions, transfers, and discharges may proceed following usual non-outbreak processes.
Mixed respiratory 	If there is a mixed viral respiratory illness outbreak with more than one pathogen • Admissions, transfers, and discharge restrictions depend on the pathogens identified. The Public Health Outbreak team will provide direction.

3.7 Transfer and discharge from an acute care facility

Determine if residents from acute care may return to the outbreak facility/unit

- Residents who were hospitalized due to illness from the outbreak pathogen may return to the facility immediately upon discharge.
 - The need for isolation is assessed based on the resident status at the time of return to the facility.
- Residents hospitalized prior or during an outbreak for an unrelated condition such as a fracture may return to the facility, based on open or restricted status.

Receiving facility/ unit status	Action
Open	The resident may return.
Restricted	- The facility follows the Risk Assessment Matrix and completes a Risk Assessment Worksheet. - Facility to coordinate with Transition Services for resident transfer and discharge.

Additional recommendations if the outbreak pathogen is influenza

Influenza 	If the outbreak pathogen is influenza. - If a resident was hospitalized due to influenza, they may return to their facility immediately upon discharge. - If a resident was hospitalized prior to the influenza outbreak or due to an unrelated condition such as a fracture, the acute care facility and the outbreak facility collaborate on the Risk Assessment Worksheet to determine if the resident may return. - Osetamivir (Tamiflu) chemoprophylaxis may be offered to the resident.
---	--

3.8 Use food service modifications in the outbreak facility/unit

- Wash hands before, during and after handling food.
- Provide meal service to residents who are on isolation in their room.
- Close resident self-service nourishment areas accessed by residents, DFSP and visitors. Regular kitchen operations may continue with HCW/staff.
- Discontinue social sharing of food such as baking, birthday cakes and platters.
- Individually portion and/or plate food to avoid communal sources. This includes removing bulk foods such as candy jars and boxes of chocolate.

Implement food service modifications as directed by the Public Health Outbreak team

Serve and prepare food safely

- Close buffet lines or have staff dispense food onto plates.
- Discontinue **family-style meal service**.
- Dispense snacks directly to residents and use prepackaged snacks.
- Discontinue resident participation in food preparation.
- Use physical distancing during group dining when possible.

Limit the use of shared items

- Do not preset tables in common dining areas to minimize contamination of items.
- Remove shared food containers from dining areas such as shared pitchers of water,

- coffee cream dispensers, and salt and pepper shakers.
- Provide single-use condiment packets directly to each resident.

3.9 Specimen collection

Collect specimens as directed by the Public Health Outbreak team

- Collect nasopharyngeal (NP) or throat swabs to identify the outbreak pathogen.
- Specimen collection is not required for all residents with symptoms.
- Notify the Public Health Outbreak team:
 - If there is a new symptom presentation among residents OR
 - If the outbreak extends beyond the original unit.

If specimen collection is requested, the Public Health Outbreak team will provide direction on:

- The number of specimens to collect
- The method of collection
- Which pathogens to test for
- The outbreak specific **exposure investigation number (EI number)** to include on the lab requisition.

Collect specimens and complete the lab requisition form

- Refer to [Appendix D: ProvLab Specimen Collection Guidance](#) for information on:
 - How to collect NP and throat swabs
 - How to complete the lab specimen requisition for viral respiratory specimens.
- Arrange for transport of specimens to the lab.

Specimen collection for individual resident medical management

- Facility may determine that specimen testing is warranted for the diagnosis and medical management of residents with symptoms including treatment with oseltamivir (Tamiflu) for influenza or Paxlovid for COVID-19.
- This testing is directed by the resident's most responsible health practitioner.

3.10 Control measures for complex outbreaks

The Public Health Outbreak team collaborates with the facility and other partners to monitor and assess each outbreak. To improve control during complex outbreaks, the Public Health Outbreak team may request the facility implement the following examples of additional outbreak control measures, which are not routine for all outbreaks.

Screening and close contact identification

- Complete symptoms screening for:
 - HCW/staff prior to each shift
 - DFSPs and visitors prior to entering the facility or visiting residents
 - Residents upon return from an absence.

- Symptom screening and/or quarantine for resident admissions upon return from other health settings if the other facility/unit is on outbreak
- **Close contact** identification and management.

Additional masking for residents

- Request residents returning from an absence wear a mask in common areas of the facility/unit.

Masking outside of the outbreak unit

- Facility-wide HCW/staff continuous masking and eye protection
- Facility-wide masking for DFSPs and visitors.

3.11 End the outbreak

The Public Health Outbreak team determines when the outbreak is over and advises the facility to discontinue restrictions and outbreak measures. After the outbreak is over, facilities will:

- Conduct a thorough cleaning and disinfection in all affected areas.
- Remove outbreak signs.
- Communicate the end of the outbreak to HCW/staff, residents, family/guardians and partners.
- Review and evaluate the outbreak response with program leads and facility management. Revise internal protocols for improvement.
 - A debrief may be requested by any member of the outbreak management team.
- Follow the steps to monitor and report viral respiratory symptoms if residents become newly symptomatic within seven days of the outbreak ending. Refer to [2.1 Monitor and report viral respiratory illness symptoms](#).

4. Identify and report gastrointestinal illness outbreaks

This section includes information on identifying and reporting GI illness outbreaks.

Refer to [Section 2. Identify and report viral respiratory illness outbreaks](#) for information on viral respiratory illness outbreaks.



Key actions

- ➔ Monitor for symptoms of GI illness in residents and HCW/staff.
- ➔ Determine if residents and HCW/staff meet criteria for a GI illness case.
- ➔ Keep track of GI illness cases.
- ➔ Report to Population and Public Health Support Team (PPHST) if there are two or more GI illness cases within 48 hours.
- ➔ Inform facility administrative staff that a report was made to PPHST.

Supporting resources

- [Outbreak Prevention Checklist for Continuing Care Homes](#)
- [Surveillance Case Tracking Sheet](#)

4.1 Monitor and report gastrointestinal illness symptoms

Monitor for symptoms

Facilities keep track of residents and HCW/staff with symptoms or lab confirmed illness (cases). This is called surveillance.

Report GI illness cases

Count residents and HCW/staff as a GI illness case if they develop at least one of the following that are not caused by something else, such as *Clostridioides difficile* diarrhea, medication, laxatives, diet, or prior medical condition:

- Two or more episodes of diarrhea (loose, or watery stools) in a 24-hour period, above what is normally expected for that individual OR
- Two or more episodes of vomiting in a 24-hour period OR
- One or more episodes of vomiting AND diarrhea in a 24-hour period OR
- One episode of bloody diarrhea OR
- Laboratory confirmation of a known enteric pathogen.

Note: Laboratory confirmation is not required.

When to report

- Report as soon as there are two or more residents and/or HCW/staff cases who have GI illness symptoms only with onset within 48 hours of each other.
- Report even if the cases are HCW/staff who were not present at work with symptoms.

How to report



Call **1-844-343-0971** to report to the PCA Provincial Public Health Support Team (PPHST).

PPHST is a provincial, centralized outbreak reporting and response team. They provide initial support and direction to facilities.

PPHST will ask for facility details and operational information

- Facility name, address, phone number
- Caller name and contact information
- Manager/facility contact email and phone number
- Facility type
- Number of units / unit names affected
- Number of residents or HCW/staff on affected units
- Number of residents or HCW/staff in facility
- Can residents be kept to affected units
- Can staff be kept to affected units
- Do staff work in multiple facilities or units
- Has facility manager been notified
- Number of symptomatic residents and HCW/staff
- Symptoms
- Onset date of first case
- Name of pathogen if lab confirmed
- Date of any hospitalizations or deaths

Inform that a report was made to PPHST

- Make internal notifications to the following facility staff after reporting to PPHST:
 - Management
 - IPC
 - WHS/OHS.

4.2 After reporting to PPHST

PPHST notifies the Public Health Outbreak team

- After the facility report is made, PPHST will:
 - Send a summary to the Public Health Outbreak team.
 - Advise facility when to expect a call from the Public Health Outbreak team.
- If the Public Health Outbreak team has not contacted the facility within 24 hours of the expected response time, the facility may call back to PPHST.

The Public Health Outbreak team contacts the facility

The Public Health Outbreak team will determine if the facility meets outbreak criteria using [Appendix C: Case and Outbreak Definitions](#). They will review the initial report and:

- Ask if there are additional GI illness cases since the initial report.
- Determine if cases are epidemiologically linked.

If an outbreak is opened, an email is sent to the facility providing ongoing contact information and instructions for how to complete daily reporting.

5. Gastrointestinal illness outbreak control

This section includes best practice recommendations for the management of GI illness⁵ outbreak. Facilities are encouraged to reach out to the Public Health Outbreak team for advice and support with the recommendations in this section.

For viral respiratory illness outbreak control, refer to [Section 3. Viral respiratory illness outbreak control](#).



Key actions

- Isolate residents with symptoms or confirmed illness.
- Perform hand hygiene.
- Inform HCW/staff, residents, family/guardians and partners of the outbreak.
- Identify and report new GI illness cases daily.
- Coordinate HCW/staff assignments.
- Complete outbreak environmental cleaning and disinfection.
- Plan safe activities for residents who are not on isolation.
- Plan for safe visits.
- Follow admission, transfer and discharge restrictions.
- Facilitate admissions and transfers from an acute care facility.
- Initiate outbreak modifications for food service.
- Collect specimens as directed.
- Manage relapse GI cases.

Supporting resources

- [Gastrointestinal Outbreak Checklist for Continuing Care Homes](#)

⁵ *Clostridioides difficile* and multi-drug resistant organisms such as MRSA and VRE can be responsible for clusters or outbreaks. Although some measures outlined in this section may be applicable in preventing or controlling them, it is beyond the scope of this guide to include these organisms due to their unique epidemiological properties.

Early detection is essential to reduce the spread of GI illness. It is vital that IPC measures are implemented immediately. Illness rates can be quite high (greater than 50%) in both residents and HCW/staff. GI illness is often mild, however residents with underlying health conditions are at risk of complications such as dehydration and aspiration pneumonia. Most outbreaks are due to norovirus which is extremely contagious.

5.1 Isolate residents with symptoms or confirmed illness

How to isolate residents

- **Diarrhea only:** Use [Contact precautions](#).
- **Vomiting with or without diarrhea:** Use [Droplet and Contact precautions](#).
- Refer to [1.3 Isolate residents with symptoms or confirmed illness](#).

Handwashing with plain soap and water is preferred during GI illness outbreaks.

If a hand hygiene sink is not available within the resident's room:

- Use alcohol-based hand rub (minimum 60-90% alcohol) prior to leaving the room.
- Then locate a sink and wash hands with plain soap and water.

How long to isolate residents

- Isolate residents until 48 hours after the last episode of vomiting and/or diarrhea.

5.2 Communicate and coordinate

Inform HCW/staff, residents, family/guardians and partners of the outbreak

- Use established communication channels to notify:
 - Residents and DFSPs/families/guardians
 - Departments/partners inside and outside of the facility.
- Request HCW/staff such as nursing, allied health, food services, environmental services, recreational services and spiritual care to initiate outbreak control measures.
- Notify laundry services of the increased need for supplies.
- Post [outbreak signs](#) at the facility/unit entrance.

Identify and report new GI illness cases daily

- Maintain heightened surveillance to identify new GI illness cases.
- Request HCW/staff complete a twice daily self-assessment for [GI illness symptoms](#).
 - Direct HCW/staff to report symptoms of GI illness to the facility/unit manager
 - Notify WHS/OHS if symptoms are related to a workplace exposure.
 - Restrict from work in all care facilities until 48 hours following the last episode

of vomiting and/or diarrhea.

- Report GI illness cases including those that are hospitalized or are deceased.
 - Review the email sent by the Public Health Outbreak team for how to report new cases.
 - The [Data Collection for Gastrointestinal Illness Outbreak Management](#) form at the end of this section outlines the information requested.

Coordinate HCW/staff assignments

- Consult with the Public Health Outbreak team when making decisions about HCW/staff assignments.
 - Direct HCW/staff to care for residents who are asymptomatic before residents with symptoms or confirmed illness.
 - Cohort HCW/staff to work only in affected areas or only in unaffected areas.
 - Minimize movement of HCW/staff between floors/areas, especially if some areas are not affected.
- Consult with facility Infection Control Practitioner (ICP) or Infection Control Designate (ICD), or the Public Health Outbreak team if considering cohorting residents.
- Assign HCW/staff to only housekeeping duties, or food preparation/service. If this is not possible, request staff complete any food preparation/service tasks before beginning housekeeping duties.
- HCW/staff to inform other facilities/units of the outbreak.
 - HCW/staff who are asymptomatic may work in the outbreak unit as well as other locations.
- Cohort volunteers to work only in affected areas or only in unaffected areas.

5.3 Complete outbreak environmental cleaning and disinfection

Increase cleaning and disinfection frequency in outbreak areas⁶

Clean and disinfect visibly dirty surfaces immediately, including all surfaces soiled with vomit or stool. Prioritize high-traffic areas.

- Follow [Public Health Recommendations for Environmental Cleaning and Disinfection of Public Facilities](#) during GI illness outbreaks.

Areas to clean and disinfect	Frequency
• Low touch surfaces such as shelves and windowsills. • Resident personal equipment such as walkers and wheelchairs.	At least once daily and when visibly dirty

⁶ Increased cleaning and disinfection is not required in settings such as private offices and administrative areas. Use routine cleaning and disinfection practices.

Areas to clean and disinfect	Frequency
- High touch surfaces such as doorknobs, light switches, handrails, phones, and elevator buttons - Care/treatment areas, including PPE carts - Equipment such as computer keyboards, mouse, screens, desks, telephones, and touch screens	At least twice daily and when visibly dirty
- Dining areas and lounges - High touch table and chair surfaces, including the underneath edge of the chair seat and table	After each use
- Upholstered furniture and rugs or carpets⁷ - Privacy curtains	When visibly dirty or contaminated with vomit or stool
- All affected areas - Privacy curtains	At the end of the outbreak

Use a disinfectant that kills GI illness viruses

Choose either:

- A disinfectant with a drug identification number that has a broad spectrum virucidal claim, or a specific virucidal claim against norovirus, feline calicivirus, or murine norovirus.
 - Refer to manufacturer instructions for product-specific cleaning and disinfection procedures, including compatible cleaners and disinfectants and contact time.
 - Use the product Safety Data Sheet for safety information.
 - An example of a product with this label claim currently in wide use in AHS/ALA facilities is 0.5% accelerated hydrogen peroxide. There are other products available with this label claim.

OR

- Use hypochlorite at a concentration of 1000 parts per million (ppm). Use commercially available hypochlorite-containing solutions (preferred).
 - Refer to manufacturer instructions for product-specific cleaning and disinfection procedures, including compatible cleaners and disinfectants.
 - Use the product Safety Data Sheet for safety information.
- Alternatively, create a 1000 parts per million bleach solution by mixing 20 mL (four teaspoons) of unscented household bleach with 1000 mL (four cups) of water.

⁷ Upholstered furniture, and rugs or carpets may be difficult to clean and disinfect completely. Follow the manufacturer instructions for cleaning and disinfection of these surfaces. If the manufacturer instructions are not available, consult the Public Health Outbreak team. Consider discarding items that cannot be cleaned and disinfected.

- Store bleach solution in an opaque container and make a fresh solution at least every 24 hours. Bleach rapidly degrades in the presence of light and when mixed with water. Label container with date and time.
- To effectively kill viruses, keep surfaces wet with the bleach water solution for at least one minute. Rinse food contact surfaces with clean water after cleaning with bleach.

Verify the concentration of the disinfectant

- When mixing disinfectants onsite, always verify the concentration using the test strips provided by the manufacturer.
- Test concentration daily.

Clean and disinfect rooms of residents who are on isolation

- Wear PPE according to the [IPCRA](#).
 - **Diarrhea only:** Use [Contact precautions](#).
 - **Vomiting with or without diarrhea:** Use [Droplet and Contact precautions](#).
- Clean and disinfect:
 - Resident room, moving from clean to dirty. Clean bathroom last.
 - Resident equipment such as wheelchairs and walkers according to manufacturer instructions.
 - Change mop head (dry/wet), cloths, and cleaning solution after:
 - Cleaning each room for any resident who is on isolation
 - After cleaning vomit or stool.
- When additional precautions are discontinued:
 - Clean and disinfect the resident's room and equipment.
 - Discard disposable resident-care items.
 - Launder unused linens such as towels and sheets from resident rooms.
 - Change privacy curtains.

Clean and disinfect shared rooms and equipment

- First clean and disinfect bedspaces of residents who are not on isolation.
- Then clean and disinfect bedspaces of residents who are on isolation.
 - Change mop head (dry/wet), cloths, and cleaning solution after cleaning each bedspace.
- Clean and disinfect shared healthcare equipment such as commodes, blood pressure cuffs, thermometers, lifts, bathtubs, showers, and shared bathrooms after use and prior to use by another resident.
- Clean and disinfect only with a product listed in the manufacturer instructions. Follow the procedures outlined by the manufacturer.

Handle linen and laundry safely

Incorporate laundry rooms into the cleaning schedule. Monitor their use, minimize storage and clutter and focus on high touch cleaning and disinfection.

Wear PPE

There is a risk of contamination of HCW/staff clothing from body fluids or secretions.

- Ensure PPE is available for use either in the laundry rooms or in an easily accessible area for use by HCW/staff doing laundry.
- Handle all linen that is soiled with body fluids. Wear PPE according to the IPCRA.
 - **Diarrhea only:** Use Contact precautions.
 - **Vomiting with or without diarrhea:** Use Droplet and Contact precautions.
- Follow correct doffing of PPE once soiled laundry is placed in the laundry bag.

Handling soiled linen and laundry safely

- Remove soiling (for example stool) with a gloved hand and dispose into a toilet at the point of care. Do not remove stool by spraying with water.
- Handle soiled laundry with minimum agitation to avoid contamination.
- Contain wet laundry before placing it in a laundry bag (for example wrap in a dry sheet or towel).
- Bag or contain soiled laundry at the point of care. It is not necessary to double bag.
- Tie laundry bags securely and do not overfill.

Keep linen and laundry used by residents on isolation separate from other laundry

- Launder clothing and linens of a resident on isolation in a designated washing machine. If using a shared resident laundry room instead of a central laundry, dedicate one washing machine to wash soiled laundry for residents on isolation.
- Do not sort or pre-rinse soiled laundry in resident care areas.
- Disinfect washer with a bleach cycle (without a load of laundry) prior to use by others if used to launder soiled items from a resident on isolation.

5.4 Plan safe activities for residents who are not on isolation

Type of activity	Recommendation
Resident activities	• Postpone/cancel all group activities. Consult with the Public Health Outbreak team if: ○ There are extenuating circumstances ○ Group activities are an essential part of treatment. • Postpone/cancel previously scheduled resident events, such as holiday meals, parties, entertainers, school groups, and presentations.
Non-resident activities	• Postpone/cancel any non-resident events booked for areas in the outbreak facility/unit such as in-person meetings.
Medical appointments	• Permit all appointments for residents who do not have symptoms. ○ Notify the receiving provider of the outbreak. ○ Virtual visits may be considered.

Type of activity	Recommendation
Transfers to acute care	- If any resident from the outbreak facility/unit requires acute medical attention or treatment at an acute care facility, notify the following so that precautions can be taken: - EMS dispatcher and/or transport staff such as the EMS crew - Receiving provider.
Adult day programs	Before determining that a program may continue, the outbreak management team (OMT) and the Public Health Outbreak team confirm the following: - It is physically separate from areas of the facility in which there have been residents with symptoms or confirmed illness. - Residents attending the adult day program do not socialize with residents from the outbreak facility/unit. - Adult day program HCW/staff do not provide care in the areas of the facility in which there have been outbreak cases. - There is a dedicated washroom available.

5.5 Plan safe visits during an outbreak

- Continue to follow safe visiting practices (refer to [1.1 Help plan a safe visit](#)).
- Provide DFSPs and visitors with safe visiting recommendations during an outbreak.

Safe visiting recommendations during an outbreak

Visiting residents who are on isolation:

- Refer to [1.1 How to safely visit residents who are on isolation](#).

Provide the following messages to DFSPs and visitors

Before the visit:

- Reschedule the visit if ill.
- Contact facility before arriving to determine if there are access limits.

During the visit:

- Perform hand hygiene.
- Check with reception prior to visiting any resident.
- Minimize movement throughout the facility.
- Limit the number of individuals visiting a resident at the same time.
- Limit visit to one resident only.

After the visit:

- Exit the facility immediately.

Additional restrictions may be implemented by the facility.

- A facility/unit may choose to limit visitation further than above in accordance with organizational policy and in consultation with the Public Health Outbreak team.
- Consider the impact of the decision on resident and family well-being.

5.6 Admission, transfer and discharge restrictions

Determine if the outbreak facility/unit is open or restricted

- The ability of the outbreak facility/unit to accept admissions, transfers and discharges (whether the unit is open or restricted) is determined by the Public Health Outbreak team in consultation with the outbreak management team at the time the outbreak is opened.
- Modifications or discontinuing of restrictions is only done by the Public Health Outbreak team.
 - If restrictions are discontinued, continue to isolate residents with symptoms or confirmed illness using additional precautions. Refer to [1.3 Isolate residents with symptoms or confirmed illness](#).
- The scope of restrictions depends on:
 - The extent of the outbreak activity within the facility (for example, one unit/floor/wing or the entire facility)
 - The ability to cohort HCW/staff to affected areas
 - The severity of the outbreak (for example, new cases continue to develop despite implemented control measures).
- Consult the Public Health Outbreak team when issues related to admission, discharge and transfers arise during an outbreak.

Status	Action
Open	• Admissions, transfers, and discharges may proceed following usual non-outbreak processes.
Restricted	• Pause or delay admissions, transfers, and discharges while restrictions are in place. ◦ If restriction of admissions/transfers is impacting the availability of acute care beds or because of the expressed informed resident or family choice, the Public Health Outbreak team will assess the circumstances surrounding the restriction including the degree of risk to the individuals requiring care.

5.7 Transfer and discharge from an acute care facility

Determine if residents from acute care may return to the facility/unit

- In extenuating circumstances, the resident return can be discussed between acute care Transition Services and the Public Health Outbreak team.
- Residents hospitalized prior to the outbreak are not recommended to return to the facility/unit until the outbreak is over.
- Consult the Public Health Outbreak team in extenuating circumstances.
 - If a transfer must occur during an outbreak, HCW/staff will collaborate with the

acute care facility prior to discharge.

- HCW/staff must advise the resident/family about the potential risks and obtain consent prior to transfer.
- Use of the Risk Assessment Matrix and/or the Risk Assessment Worksheet is based on Zone process. Use if directed by the Public Health Outbreak team.

5.8 Use food service modifications in the outbreak facility/unit

- Use stringent hand hygiene.
- Provide meal service to residents who are on isolation in their room.
- Close resident self-service nourishment areas accessed by residents, DFSP and visitors. Regular kitchen operations may continue with HCW/staff. Discontinue social sharing of food such as baking, birthday cakes and platters.
- Use individually portioned and/or plate food to avoid communal sources. This includes removing bulk self-serve foods such as candy jars and boxes of chocolate.
- Use extra diligence during routine dishwashing and during sanitizing practices for food preparation surfaces.

Implement setting specific food service modifications as directed by the Public Health Outbreak team

Prepare and serve food safely.

- Discontinue resident participation in food preparation.
- Close buffet lines or have staff dispense food onto plates.
- Discontinue family-style meal service.
- Dispense snacks directly to residents and use prepackaged snacks.
- Use dining table coverings that can be easily cleaned and disinfected.
 - Discontinue use of cloth/linen table coverings.

Limit the use of shared items

- Do not preset tables in common dining areas to minimize contamination of items.
- Remove shared food containers from dining areas such as shared pitchers of water, coffee cream dispensers, and salt and pepper shakers.
- Provide single-use condiment packets directly to each resident if used.

5.9 Specimen collection

Collect specimens as directed by the Public Health Outbreak team

- Stool specimens are collected to identify the pathogen causing the outbreak.
- Specimen collection is not required for all residents with symptoms.
- Notify the Public Health Outbreak team:
 - If there is a new symptom presentation among residents, OR
 - If the outbreak extends beyond original the unit.

If specimens are requested the Public Health Outbreak team will provide direction

- The number of specimens to collect
- Which pathogens to test for, including specific instructions on which pathogen to check off on the lab requisition form
- The outbreak specific exposure investigation number (EI number) to include on the lab requisition.

Collect specimens and complete the lab requisition form

- Refer to [Appendix D: ProvLab Specimen Collection Guidance](#) for information on:
 - How to complete the lab specimen requisition for stool specimens
 - How to collect stool specimens.
- Arrange for transport of specimens to the lab.

After initial specimen collection

- Do not collect additional specimens unless directed by the Public Health Outbreak team. Usually, once a pathogen is identified no more stool specimens are requested to manage the outbreak.
- ProvLab will only test additional specimens after consultation with the Public Health Outbreak team.
- The Public Health Outbreak team will contact the ProvLab if the clinical situation for the outbreak has changed and additional testing needs to be done.

5.10 Control measures for complex outbreaks

The Public Health Outbreak team collaborates with the facility and other partners to monitor and assess each outbreak. To improve control during complex outbreaks, the Public Health Outbreak team may request the facility implement the following additional outbreak control measures, which are not routine for all outbreaks.

- Symptom screening of HCW/staff prior to each shift.
- Symptom screening of DFSPs and visitors prior to entering the facility or visiting residents.
- Symptom screening of residents upon return from an absence.
- Symptom screening and/or quarantine for resident admissions upon return from other health settings if that other facility is on outbreak.

5.11 How to manage relapse gastrointestinal illness cases

GI illness cases frequently relapse. That is, they experience onset of vomiting or diarrhea after being asymptomatic for up to 48 hours.

- The relapse is likely due to malabsorption during an existing norovirus infection rather than being a new infection.

If a relapse GI illness case is identified:

- Isolate resident in their room until free of vomiting and/or diarrhea for 48 hours. Refer to [1.3 Isolate residents with symptoms or confirmed illness](#).
- Do not count as a new outbreak case if the relapse is within seven days of original symptom resolution.
 - Relapse cases are not included on new daily case listings.
 - Relapse case(s) alone will not extend admission and transfer restrictions.
- If a previously identified GI illness case has onset of GI illness symptoms after being symptom free for at least seven days, manage as a new case.

5.12 End the outbreak

The Public Health Outbreak team determines when the outbreak is over and advises the facility to discontinue restrictions. After the outbreak is over, facilities will:

- Conduct a thorough cleaning and disinfection in all affected areas.
- Remove outbreak signs.
- Communicate the end of the outbreak to HCW/staff, residents, family/guardians and partners.
- Review and evaluate the outbreak response with program leads and facility management. Revise internal protocols for improvement.
 - A debrief may be requested by any member of the outbreak management team.
- Follow the steps to monitor and report GI illness symptoms if residents become newly symptomatic within seven days of the outbreak ending. Refer to [4.1 Monitor and report gastrointestinal illness symptoms](#).

Data Collection for Gastrointestinal Illness Outbreak Management

The Public Health Outbreak team will direct facilities on how to report when the GI illness outbreak is opened. The individual responsible for completing and submitting the list of cases is facility specific. Reporting may be done by facility ICP/ICD, facility/unit manager or another responsible HCW/staff in the facility/unit.

Report the following data daily to the Public Health Outbreak team:

- **Outbreak facility/site** (name, unit/floor, contact person, phone, and fax)
- **Date of report**
- **Population affected at the time outbreak is reported** (total resident and HCW/staff population at risk on the outbreak unit/site, number of residents and HCW/staff who meet the case definition)
- **Outbreak/ exposure investigation number (EI)** (as provided by the Public Health Outbreak team)
- **Demographics of cases**
 - Residents: name, personal health number, date of birth, gender, unit/room number
 - HCW/staff: number of new cases
- **Signs and symptoms**
 - Onset date
 - Signs and symptoms meeting case definition (vomiting, diarrhea, bloody diarrhea)
- **Lab tests/results**
 - Stool specimen (date sent)
 - Results
- **Hospitalization or death of cases due to the outbreak**
 - Cases hospitalized (name, personal health number, date of admission, name of hospital)
 - Cases who died (name, personal health number, date, and cause of death)

Glossary

Acute care: Includes all urban and rural hospitals, psychiatric facilities, and urgent care facilities where inpatient care is provided.

Adult day program: Day program designed for adults with physical and/or memory challenges or chronic illness.

Appropriate mask: The type of mask such as medical, KN95, N95 recommended per the AHS IPC [Infection Prevention and Control Risk Assessment \(IPCRA\)](#).

Close contact: Any person suspected to have been exposed to an infected person or a contaminated environment to a sufficient degree to have had the opportunity to become infected or colonized with a pathogen.

Cluster: A grouping of similar, relatively uncommon events or diseases in space and/or time in amounts that are believed or perceived to be greater than could be expected by chance.

Cohorting: Controlling the movement of HCW/staff and residents for the purpose of limiting an outbreak to a specific unit/floor/area within a larger facility. The physical separation of people who have been or might have been exposed to infection from those who have not been exposed.

Continuing care home: Continuing care home means a facility or part of a facility where facility-based care is provided to residents, some of whom must be eligible residents. Continuing care homes are publicly funded facility-based accommodations that provide care (health and support services) appropriate to meet the resident's assessed needs. The type of care needed is determined through a standardized assessment and single point of entry process and consists of Type A, Type B and Type C. Individuals admitted into Continuing Care Homes Type A and B are required to pay accommodation fees (room and board and other costs associated) as set by Alberta Primary and Preventative Health Services (PPHS).

Continuing Care Home Type A (formerly long term care): This environment provides onsite registered nurse (RN) and/or registered psychiatric nurse (RPN) care, assessment and/or treatment 24-hours a day. Licensed practical nurses (LPNs) may also be onsite. In addition, onsite personal care and support provided by healthcare aides (HCAs). CCH Type A may also have a secure space. Some facilities may have specialized programs and services available for residents with complex clinical or complex functional care requirements (e.g., rehabilitation).

Continuing Care Home Type B (formerly designated supportive living): This environment provides a purposeful home-like design with 24-hour a day onsite scheduled and unscheduled professional and personal care provided by LPNs and HCAs. Case management and specialty services (e.g., Allied Health, palliative resource nurse, etc.) are available on a scheduled onsite, on-call or virtual basis based on resident's care needs. The buildings are specifically designed with common areas and features, including private space and a safe, secure and barrier-free environment. Type B facilities promote independence and provides services such as meals, housekeeping, recreational activities and 24-hour monitoring.

Continuing Care Home Type B – secure space (formerly designated supportive living dementia): This environment provides a purposeful home-like design with small groupings of private bedrooms and associated spaces with security features (i.e., secured spaces).

Continuing Care Home Type C (hospice): Hospice care is a specialized service that provides 24/7 care to individuals who are approaching end-of-life and their families. Services are available for both adult and pediatric populations and may include respite. The care teams focus on your comfort and quality of life and can support you and your family throughout the last months, weeks, or days of life. Services are provided by a specialized interprofessional palliative care team that includes (but is not limited to): doctors, registered nurses, licensed practical nurses, social workers, and healthcare aides. Availability of other professionals such as spiritual counsellors and other services such as volunteers may be different for each hospice.

Designated family / support person(s) (DFSPs): individual(s) identified by the resident as an essential support, and who they want included in care planning and decision making. This includes but is not limited to family, relatives, friends, and informal or hired caregivers. Refer to [Family/Visitors of Patients and Residents](#).

Exposure investigation number (EI number): A number assigned by the ProvLab to track laboratory specimens associated with an outbreak at a specific location and time.

Facility operator / facility management: A formal leader within the facility who is responsible for the day-to-day operations.

Family-style meal service: Involves filling a common vessel, such as a tray or bowl, with a large portion of food and setting it on the table allowing residents to serve themselves.

Healthcare workers (HCW): As defined by Alberta Primary and Preventative Health Services (PPHS) includes all health practitioners and all individuals (including nutrition and food services, housekeeping and recreation) at increased risk for exposure to, and/or

transmission of, a communicable disease because they work, study, or volunteer in one or more of the following healthcare settings: hospitals, continuing care homes, supportive living accommodations or home and community care settings, mental health facilities, community settings, offices or clinics of a health practitioner, clinical laboratories.

Healthcare workers / staff (HCW/staff): For the purpose of this guide, the term HCW/staff is used consistently. The facility is responsible to determine if an individual is considered a HCW or a staff member. The facility determines when actions need to be taken by all staff members at the facility such as administrative staff, support staff, and regulated healthcare providers or only those that are considered to be a HCW such as healthcare aide, licensed practical nurse, registered nurse, professional and non-professional therapies/Allied Health).

- Onsite Staff: HCW/staff that in some capacity of their job work directly at that facility. When an individual is onsite, they are expected to fulfill the roles and responsibilities of onsite HCW/staff.
 - Staff employed directly by the facility
 - Staff who are contracted service providers and work at the facility (for example contracted pharmacy services)
 - AHS and ALA staff who work and provide services at the facility.

Infection control designate (ICD): Person accountable for IPC issues in a facility usually, facility based.

Infection control professional (ICP): A health professional with specialized knowledge responsible for infection prevention and control within the facility or area of practice. ICPs come from several disciplines, including nursing, medicine, microbiology, medical technology and/or epidemiology and may be certified or working toward certification in infection control (CIC®).

Infection Prevention & Control (IPC): Department responsible for preventing infections acquired within facility. IPC includes roles such as consultation, education, surveillance, research and outbreak management.

Most responsible health practitioner: The healthcare provider who has responsibility and accountability for the specific treatment/procedure(s) provided to a resident and who is authorized by AHS, ALA and/or the facility to perform the duties required to fulfill the delivery of such a treatment/procedure(s) within the scope of their practice.

Outbreak management team (OMT): A group of key individuals working cooperatively to ensure a timely and coordinated response to an outbreak. Membership on the OMT depends on the type of outbreak and facility. Membership may include representatives

from the Public Health Outbreak team, Infection Prevention and Control (IPC or ICD), Workplace Health and Safety / Occupational Health and Safety, facility administration / facility management or their designate. Any member may request an OMT is formed.

Outbreak control measures: Actions to control the spread of disease during an outbreak. Outbreak control measures may fall into the following categories:

- Routine measures: measures for all viral respiratory or GI illness outbreaks.
- Pathogen-specific measures: measures specific to the outbreak pathogen.
- Control measures for complex outbreaks: measures that are not routinely used. They are implemented only at the discretion of the Public Health Outbreak team if the outbreak warrants additional control measures.

Plain soap: detergent based products whose cleansing activity can be attributed to their detergent properties which remove organic substances from hands. These products have minimal, if any, antimicrobial activity.

Public Health Outbreak team: This team is made up of Medical Officers of Health (MOH), the Communicable Disease Control (CDC) Nurses and Safe Healthy Environments Public Health Inspectors. It provides consultation and leadership in outbreak investigations in facilities and reports outbreaks to Alberta Primary and Preventative Health Services (PPHS).

Visitor: An individual who temporarily supports or socializes with the patient. They are not an essential partner in care planning and/or decision-making. For more information refer to the AHS [Family Presence: Designated Family / Support Person and Visitor Access policy](#).

Workplace Health and Safety / Occupational Health and Safety (WHS/OHS): Designated personnel responsible for HCW/staff health and safety in facilities. This role may be filled by HCW/staff or the facility management.

Work restriction: A measure that prevents HCW/staff with symptoms, who are infected or susceptible from working until the risk for residents or healthcare worker/staff is low or minimal.

Appendix A: Roles and Responsibilities

This appendix outlines roles and responsibilities to prevent and manage outbreaks⁸. Due to the complex nature of staffing and resident populations in continuing care homes, the individual fulfilling the roles and responsibilities within a facility may vary from what is outlined below.

An outbreak management team (OMT) may be formed during an outbreak to facilitate a timely and coordinated response to the outbreak. Any of the groups identified below may request that an OMT be formed.

Public Health Outbreak team	
Prior to the outbreak	Plan • Develop, maintain, and distribute provincial outbreak resources. • Participate in fall education sessions. Consult • Consult on suspected clusters of illness, symptoms or potential outbreaks. • Determine if an outbreak will be opened.
During the outbreak	Support • Recommend best practice outbreak control measures. • Obtain a ProvLab EI number and provide to the facility. • Direct outbreak specimen collection. • Track and assess outbreak specimen results. • Review daily outbreak data and monitor outbreak progress. Consult • Provide consultation on outbreak control measures such as: ○ Resident isolation ○ Masking and eye protection for HCW/staff for viral respiratory illness outbreaks. ○ HCW/staff management ○ Oseltamivir (Tamiflu) for all residents and HCW/staff who are not immunized during influenza outbreaks. ○ Outbreak environmental cleaning ○ Group activities ○ Safe visiting of residents ○ Restrictions to admissions, transfers and discharges ○ Food service modifications

⁸ The roles and responsibilities of Alberta Primary and Preventative Health Services (PPHS), ALA Zone Operations (Executive Director for Provincial Seniors Health, Area Managers / Directors) and ALA Provincial Seniors Health and Continuing Care are not discussed in this guide. For information about the roles and responsibilities of these partners, contact these groups directly.

- Discuss facility-specific adaptations to outbreak control measures.
- Direct control measures for complex outbreaks.
- Report outbreak to Alberta Primary and Preventative Health Services (PPHS).
- Participate in OMT meetings.
- Declare the end of the outbreak.

PCA Provincial Partner Oversight team

Prior to the outbreak

- Support continuing care home operators to provide access to outreach immunization services for residents and staff.
- Report data such as immunization provider, number of residents and staff to facilitate vaccine allocations to Alberta Primary and Preventative Health Services (PPHS).
- Share regular communication such as monthly newsletters and [PPO webpage](#).

Facility administration / management or designate

Prior to the outbreak

Plan and educate

- Refer to *Minimum Requirements of a Comprehensive Communicable Disease Emergency Response Plan* in [Continuing Care Communicable Disease Emergency Response Guide](#) to develop a facility outbreak management plan.
 - Including surveillance, isolation, process for specimen collection and immunization for residents and HCW/staff.
 - Develop an influenza antiviral chemoprophylaxis plan for residents and HCW/staff.
- Provide an annual update on outbreak prevention and control for facility HCW/staff. Include review of:
 - The facility outbreak management plan
 - The Guide for Outbreak Prevention & Control in Continuing Care Homes
 - When and how to report to PPHST.

Immunization

- Facilitate immunization of residents and HCW/staff, including COVID-19 and influenza.

Influenza antiviral chemoprophylaxis

- Provide the [Advance Prescription for Oseltamivir \(Tamiflu\)](#) fillable form to residents for completion by their most responsible health practitioner.
 - Track residents with advanced prescriptions using the [Outbreak Antiviral Prophylaxis in Supportive Living Accommodation and Continuing Care Homes Type B Sites Worksheet](#).
 - Develop policy outlining who is responsible for the cost of HCW/staff chemoprophylaxis.

Supplies

- Stock non-expired outbreak management supplies including PPE and specimen collection kits.

	IPC • Direct the implementation of routine practices and additional IPC measures. • Notify IPC or ICD when an unusual cluster of illness/symptoms is suspected or a report has been made to PPHST. • Log in to Continuing Care Connection for ongoing practice guidance updates relevant to continuing care homes. Registration is required the first time the website is accessed.
During the outbreak	Collaborate • Collaborate with IPC or ICD and the Public Health Outbreak team on outbreak control measures. • Collaborate with WHS/OHS designate to identify and report HCW/staff with symptoms who may have been exposed at work. • Consult with the Public Health Outbreak team on issues pertaining to admission, discharge, and transfers. • Participate in OMT meetings. Communicate • Communicate about the outbreak. • Communicate the status of the facility (open or restricted) to partners. Coordinate • Coordinate specimen collection as directed by the Public Health Outbreak team. • Distribute to HCW/staff letters provided by the Public Health Outbreak team. • Collect and report HCW/staff immunization status. Communicate during an influenza outbreak that influenza antiviral chemoprophylaxis for HCW/staff is not publicly funded. • Anticipate and manage the impact of HCW/staff absence on facility operations.

IPC, including AHS Infection Control Professional (ICP) and facility Infection Control Designate (ICD)

The following tasks are generally managed by the facility ICD. In the absence of a facility ICD, administration / manager or designate assigns responsibility for these roles.

This role may be fulfilled by a single individual, or by a combination of ICD and supporting groups such as organizational/zone/AHS or Covenant ICP.

AHS/Covenant ICP is available to assist and guide where needed.

Prior to the outbreak

Plan

- Update internal outbreak management resources and review with HCW/staff, including routine IPC practices and additional precautions.
- Maintain ongoing surveillance and support HCW/staff to facilitate early recognition and reporting of residents with symptoms or confirmed illness (and HCW/staff for GI illness symptoms).
- Ensure HCW/staff have access to and are familiar with this guide (Guide for Outbreak Prevention and Control in Continuing Care Homes).
- Collaborate with the Public Health Outbreak team to provide fall education sessions.

PPE

- Encourage adoption of the PPE Safety Coach program.
- Liaise with WHS/OHS designate to ensure HCW/staff have been N95 respirator fit tested.

IPC

- Direct IPC practices for residents with symptoms or confirmed illness to prevent further transmission.

During the outbreak

Collaborate

- Consult with the Public Health Outbreak team.
- Identify high risk activities that need to be postponed or cancelled in consultation with the Public Health Outbreak team.
- Collaborate with facility administration / manager or designate to coordinate specimen collection.
- Participate in OMT meetings.

Coordinate

- Coordinates daily submission of outbreak data.
- Implement outbreak control measures in consultation with facility Medical Lead.

IPC

- Consult on IPC measures.
- Complete facility outbreak reviews and outbreak visits (AHS ICP).
- Facility ICD to connect with AHS / Covenant Health IPC.

Workplace Health and Safety / Occupational Health and Safety / designate

In the absence of formal AHS WHS or facility OHS coverage, facility administration/ manager designates responsibility for these roles.

Prior to the outbreak

Plan

- Facilitate immunizations for HCW/staff, including COVID-19 and influenza in collaboration with the PPO Team.
- Reviews internal processes for HCW/staff outbreak management.
- Arrange HCW/staff N95 respirator fit testing.

During the outbreak

Collaborate

- Support facility administration / manager or designate to identify HCW/staff exposures.
- Participates in OMT meetings.

Communicate

- Communicates with the Public Health Outbreak team if HCW/staff are linked to the outbreak.

HCW/staff immunization status

- Document HCW/staff health and immunization status
- Provide work restriction recommendations during influenza outbreaks to the Facility/Unit Manager based on immunization status.
- Identifies HCW/staff who are not immunized and who may be at risk of exposure and infection.
- Implement facility plan for HCW/staff who are not immunized to access and pay for oseltamivir (Tamiflu) chemoprophylaxis.

Onsite HCW/staff

Prior to the outbreak

Plan

- Review this guide (Guide for Outbreak Prevention and Control in Continuing Care Homes).
- Use PPE as per the Infection Prevention and Control Risk Assessment (IPCRA) prior to providing resident care.
- Identify residents with new symptoms or confirmed illness through ongoing surveillance.
 - Place on additional precautions.
 - Assess if residents with symptoms or confirmed illness meet the reporting criteria to PPHST.

During the outbreak	Plan - Identify residents with new symptoms or confirmed illness through ongoing surveillance. - Place on additional precautions. Collaborate - Collect specimens as directed by the Public Health Outbreak team. - Submit outbreak data daily to the Public Health Outbreak team. - Participate in OMT meetings. Communicate - Communicate outbreak status to other facility programs that are impacted by the outbreak such as Adult Day Programs and child care. - Report PPE and hand hygiene breaches. WHS/OHS - Comply with work restrictions. - Report relevant immunization status.
----------------------------	--

Public Health Laboratory (ProvLab)

Prior to the outbreak	Provide specimen collection supplies.
During the outbreak	- Designate laboratory microbiologist or virologist for each outbreak. - Track samples submitted under the EI number. - Report outbreak specimen results. - Complete additional testing on outbreak specimens.

Appendix B: Resident Isolation for Viral Respiratory Illness

Duration of isolation

- Isolate residents with symptoms or confirmed viral respiratory illness⁹ on [Droplet and Contact precautions](#).
- Isolation ends and Droplet and Contact precautions are discontinued when:
 1. Symptoms have improved
and
 2. Resident feels well enough to resume normal activities
and
 3. Resident is fever-free for 24 hours without the use of fever-reducing medication.
- [Isolation duration will vary](#) depending on how long it takes for the resident to meet the three conditions above. When isolation ends:
 - Discontinue Droplet and Contact precautions.
 - Complete a post isolation cleaning and disinfection of the resident's room.
 - Remove isolation supplies and signage.

Added precautions after isolation ends

- After isolation ends, [for the next five days](#) residents are recommended to use these added precautions¹⁰:
 - Clean hands well and often.
 - Wear a mask in common areas in the facility and in public spaces.
 - Practice physical distancing if removing mask, such as during meals.
- Rationale for added precautions: Residents without symptoms are much less likely to spread viral respiratory illness than those with symptoms. Residents who feel well enough to resume normal activities are unlikely to spread illness to others, but it may still be possible.

⁹ Viral respiratory illnesses include Adenovirus, COVID-19, Enterovirus/Rhinovirus, Human Metapneumovirus (hMPV), Influenza, Non-COVID-19 Coronaviruses, Parainfluenza, Respiratory Syncytial Virus (RSV), viral respiratory illness [case](#) (no pathogen identified)

¹⁰ Facilities are encouraged to use the [help prevent the spread of respiratory illness poster](#) in discussions with residents and families about the rationale for added precautions after isolation ends.

Appendix C: Case and Outbreak Definitions

Case and outbreak definitions are set by Alberta Primary and Preventative Health Services (PPHS) and are used to open and report outbreaks.

COVID-19

Case definition

A person with the virus (SARS-CoV-2) that causes COVID-19 by:

- A positive result on a molecular test [that is Nucleic acid amplification test (NAATs) such as polymerase chain reaction (PCR)], loop-mediated isothermal amplification (LAMP) or rapid molecular test] that is Health Canada approved or approved by the lab accreditation body of the jurisdiction in which the test was performed.

OR

- A positive result on a Health Canada approved rapid/point-of-care (POC) antigen test in a person with clinical illness¹¹

OR

- Two positive results on a Health Canada approved rapid/POC antigen test completed not less than 24 hours of each other in a person who is asymptomatic.

Outbreak definition

Two or more confirmed COVID-19 cases in residents within a seven-day period, with a common epidemiological link¹².

Outbreak duration¹³

14 days (two incubation periods). The outbreak ends on the 15th day following symptom onset of the last resident case.

¹¹ Clinical illness - any one or more of the following: new or worsening cough, shortness of breath (SOB), sore throat, loss or altered sense of taste/smell, runny nose/nasal congestion, fever/chills, fatigue (significant and unusual), muscle ache/joint pain, headache, nausea/diarrhea).

¹² Epidemiological link means the cases need to have been in the setting (same facility/same unit) during their incubation period or communicable period.

¹³ Day zero is the first day of symptoms. Day one is the first full day after symptoms develop.
If the person tested is asymptomatic, use date of specimen collection as day zero.

Influenza

Case definition	A person with clinically compatible signs and symptoms of syndromic ILI¹⁴ and laboratory confirmation of infection with seasonal influenza virus by: • Detection of influenza virus RNA such as via real-time reverse transcriptase polymerase chain reaction (RT-PCR) OR • Demonstration of influenza virus antigen in an appropriate clinical specimen such as nasopharyngeal/throat swabs OR • Significant rise (that is fourfold, or greater) in influenza IgG titre between acute and convalescent sera OR • Isolation of influenza virus from an appropriate clinical specimen
Outbreak definition	Two or more confirmed influenza cases in residents within a seven-day period, with a common epidemiological link ¹² .
Outbreak duration ¹³	Seven days. The outbreak ends on the eighth day following symptom onset of the last resident case.

¹⁴ **Syndromic ILI** is acute onset of viral respiratory illness which includes cough (new or worsening) and one or more of the following symptoms: fever, shortness of breath, sore throat, myalgia or arthralgia.

Influenza-like illness (ILI)

Case definition	Syndromic ILI: • Acute onset of viral respiratory illness which includes cough (new or worsening) and one or more of the following symptoms: ○ Fever¹⁵ ○ Shortness of breath ○ Sore throat ○ Myalgia ○ Arthralgia ○ Prostration Pathogen-specific ILI: • Positive for non-influenza, non-COVID-19 pathogen from the Respiratory Pathogen Panel (RPP)
Outbreak definition	Two or more cases of ILI in residents within a seven-day period, with a common epidemiological link¹² AND No viral respiratory pathogen identified OR only one case of any viral respiratory pathogen identified such as Influenza, COVID-19 and RSV OR at least two cases of a non-influenza, non-COVID-19 respiratory pathogen. Note: ILI outbreaks can be either syndromic or pathogen specific. • Examples of syndromic ILI outbreaks include: ○ Two or more residents who meet the syndromic case definition OR ○ One resident who meets the syndromic case definition PLUS at least one other resident who meets a pathogen-specific case definition. • Examples of pathogen-specific ILI outbreaks include: ○ Two or more residents with the same non-influenza, non-COVID pathogen from the RPP.
Outbreak duration¹³	• If no pathogen is identified: ILI outbreak remains open for seven days. The outbreak ends on the eighth day following symptom onset of the last resident case. • If a non-influenza, non-COVID-19 respiratory pathogen is identified: the outbreak remains open for a single incubation period for that pathogen.

¹⁵ In people 65 years and older, fever may not be prominent.

Pathogen	Outbreak Duration
Adenovirus	14 days (a single incubation period). The outbreak ends on the 15 th day following symptom onset of the last resident case.
Enterovirus Rhinovirus	Three days (a single incubation period). The outbreak ends on the fourth day following symptom onset of the last resident case.
Human Metapneumovirus	Five days (a single incubation period). The outbreak ends on the sixth day following symptom onset of the last resident case.
Non-COVID-19 Coronavirus	Four days (a single incubation period). The outbreak ends on the fifth day following symptom onset of the last resident case.
Parainfluenza Type 1, 2, 3, 4	Six days (a single incubation period). The outbreak ends on the seventh day following symptom onset of the last resident case.
Respiratory Syncytial virus	Eight days (a single incubation period). The outbreak ends on the ninth day following symptom onset of the last resident case.

Gastrointestinal illness

Case definition	At least ONE of the following criteria must be met and not be attributed to another cause (such as <i>Clostridioides difficile</i> diarrhea, medication, laxatives, diet, or prior medical condition): - Two or more episodes of diarrhea (loose, or watery stools) in a 24-hour period, above what is normally expected for that individual OR - Two or more episodes of vomiting in a 24-hour period OR - One or more episodes of vomiting AND diarrhea in a 24-hour period OR - One episode of bloody diarrhea OR - Laboratory confirmation of a known enteric pathogen Note: Lab confirmation is not required.
Outbreak definition	Two or more cases (with initial onset within one 48-hour period) of GI illness with a common epidemiological link ¹² .
Outbreak duration	Outbreak duration may vary. The Public Health Outbreak team determines outbreak duration. - Generally outbreaks are ended according to either timeframe below (whichever comes first): 48 hours from symptom resolution in the most recent case OR 96 hours from onset of symptoms in the most recent case.

Mixed viral respiratory pathogen and mixed pathogen outbreaks

A **mixed viral respiratory pathogen outbreak** may result when a combination of lab positive respiratory pathogens/viruses are identified. Similarly, a **mixed pathogen outbreak** could result when virus(es) causing viral respiratory and GI illness symptoms are co-circulating in a facility.

Outbreak duration	The Public Health Outbreak team determines if the facility has a mixed pathogen outbreak. The general principle of applying the more protective outbreak control measures will be followed.
--------------------------	---

Note: During mixed viral respiratory and GI illness outbreaks, the Public Health Outbreak teams from Communicable Disease Control and Safe Healthy Environments collaborate with the facility and each other to control the outbreaks. Processes may vary by zone.

Appendix D: ProvLab Specimen Collection Guidance

Resources for specimen collection

- Check the [ProvLab Bulletins](#) for the most current information on specimen collection, testing, and interpretation of lab results.
- Use the [Lab Supply Ordering Portal](#) to order specimen collection supplies.
- Use [Education Resources](#) from the Public Health Laboratory (ProvLab) for instructions and demonstrations on specimen collection.

Respiratory specimens

Collect respiratory specimens for viral respiratory illness outbreak management when directed by the Public Health Outbreak team.

How to complete the lab specimen requisition for viral respiratory specimens

- Complete [COVID-19 and Other Respiratory Viruses Requisition \(Provincial\)](#)

Follow directions in the [General Test Ordering and Specimen Collection Information – Clinical Specimens](#) to complete the requisition.

- Include the Exposure Investigation Number (EI Number) provided by the Public Health Outbreak team for the current outbreak.
- Use the Most Responsible Health Practitioner as the **Authorizing Provider Name**. Do not use the Zone MOH.

How to collect nasopharyngeal (NP) and throat swabs.

- The amount of virus is greatest in acute phase of illness, usually within the first 48-72 hours of symptom onset. This is the optimal window for specimen collection.
- NP swabs are the preferred specimens for viral respiratory testing.
 4. Refer to [Collection of a Nasopharyngeal and Throat Swab for Detection of Respiratory Infection](#).
 5. If NP swabs are difficult to collect, throat swabs collected in viral transport media are acceptable alternatives for COVID-19 testing. A Respiratory Pathogen Panel (RPP) cannot be completed on a throat swab.

GI specimens

Collect stool specimens for GI illness outbreak management when directed by the Public Health Outbreak team.

- Norovirus can be detected in stool by ordering a Gastroenteritis Viral Panel (LAB 1901) but cannot be isolated from vomit. Do not collect vomit specimens.

- An EI number is required on the requisition or norovirus testing may not be completed in the analysis. Obtain an EI number from the Public Health Outbreak team prior to collecting outbreak stool specimens.
- Results are usually available on Netcare within 48 hours. The Public Health Outbreak team will report the result to the ICP/ICD/designate within one business day of receipt of results from the lab.

How to complete the lab specimen requisition for stool specimens.

- Complete the [Public Health Microbiology Requisition](#)
- Follow direction from the Public Health Outbreak team on which pathogens to test for including specific instructions on which pathogen to check off on the lab requisition form

Follow directions in the [General Test Ordering and Specimen Collection Information – Clinical Specimens](#) to complete the requisition.

- Include the EI number provided by the Public Health Outbreak team for the current outbreak.

How to collect stool specimens.

- Follow instructions in [Enteric Bacterial Panel and C. difficile Stool Collection Patient Instructions](#).
- Collect stool specimens from residents with acute diarrhea illness. Preferably, collect within 24-48 hours of onset of symptoms.
- When directed, collect one stool specimen from up to five residents with symptoms per outbreak investigation (EI number). This number of specimens is usually sufficient to determine the etiology of the outbreak

Transport specimens to ProvLab.

- Facilities are responsible for arranging transport of specimens.
- Follow ProvLab standards for transporting specimens at [Laboratory Test Directory & Collection Information](#).
- **Urban facilities:** Batch together and transport specimens to the ProvLab within 24 hours of collection.
- **Rural facilities:** Transport lab specimens to ProvLab as directed by the Public Health Outbreak team or by the fastest means possible.
- AHS/ALA managers and staff can access the My Learning Link Transportation of Dangerous Goods (TDG) Class 6.2 Infectious Substances modules on safe specimen transport. If staff member does not have access to My Learning Link, connect with manager to determine where this learning can be accessed.

Appendix E: Influenza Outbreak Work Restrictions

The Public Health Outbreak team will provide recommendations for immunization, antiviral chemoprophylaxis and/or work restrictions for HCW/staff during influenza outbreaks. Oseltamivir (Tamiflu) antiviral chemoprophylaxis is recommended according to the Alberta Primary and Preventative Health Services (PPHS) Influenza Antiviral Drug Policy. Facilities are to identify HCW/staff who are recommended to take antiviral chemoprophylaxis and advise them how to access it.

For HCW/staff with symptoms, manage as per [HCW/staff with symptoms](#).

Assessment to determine if HCW/staff who are asymptomatic may work on the outbreak unit

HCW/staff are considered to be **adequately protected** against influenza if:

- They have had the annual influenza vaccine **AND**
- It has been at least 14 days since influenza immunization.

HCW/staff who are asymptomatic who were immunized at least 14 days prior to the outbreak

- HCW/staff immunized 14 days prior are considered to have adequate protection against influenza.
 6. There is no work restriction. HCW/staff may continue to work on the outbreak facility/unit.
 7. Antiviral chemoprophylaxis is not recommended.

HCW/staff who are asymptomatic without adequate protection against influenza

Refer to the table below for work restriction and antiviral chemoprophylaxis recommendations for HCW/staff who are:

- Immunized with annual influenza vaccine less than 14 days ago **OR**
- Not immunized with annual influenza vaccine
 8. Offer influenza vaccine immediately to HCW/staff who are not immunized.

Antiviral chemoprophylaxis and work restriction recommendations

HCW/staff who are asymptomatic	Accepts antiviral chemoprophylaxis	Does not accept antiviral chemoprophylaxis
Immunized with annual influenza vaccine less than 14 days ago	- May continue to work on the outbreak facility/unit AND - Take antiviral chemoprophylaxis until at least 14 days since immunization or until the outbreak is over (whichever is shorter). - There is no waiting period between starting antiviral chemoprophylaxis and working.	First 3 days (72 hours) Restricted from working at any facility and will monitor for symptoms for 3 days (72 hours) from last day worked on outbreak facility/unit.
		Day 4 to end of outbreak if remains asymptomatic May be reassigned to a non-outbreak facility/unit for at least 14 days since immunization or until the outbreak is over (whichever is shorter). - May then return to home facility/unit. If reassignment is not possible restrict from work for at least 14 days since immunization or until outbreak is over (whichever is shorter). - May then return to work.
Not immunized with annual influenza vaccine	- May continue to work on the outbreak facility/unit if they take antiviral chemoprophylaxis until the outbreak is over. - There is no waiting period between starting antiviral chemoprophylaxis and working.	First 3 days (72 hours) Restricted from working at any facility and will monitor for symptoms for 3 days (72 hours) from last day worked on outbreak facility/unit.
		Day 4 to end of outbreak if remains asymptomatic May be reassigned to a non-outbreak facility/unit for the duration of the outbreak. - May then return to home facility/unit. If reassignment is not possible restrict from work until outbreak is over. - May then return to work.